

CASE NO. 25-CI-002964

JEFFERSON CIRCUIT COURT
DIVISION FOUR (4)
JUDGE JULIE KAE LINCHARLIE TINKER, Individually and as Executor
of the ESTATE OF THELMA TINKER, Deceased,
and DENISE HOUCK, MONICA RUSSELL, BRIAN
TINKER, AND CHERI KALILIKANI, Each Individually

PLAINTIFFS

v.

UofL HEALTH–LOUISVILLE, INC. d/b/a UofL HEALTH;
UNIVERSITY OF LOUISVILLE PHYSICIANS, INC.;
ERICA GETTIS, D.O.; ROBERT ARIA, M.D.; and
UofL HEALTH, INC.

DEFENDANTS

*** **

NOTICE – MOTION – ORDER

Please take notice that on **Monday, June 1, 2026, at 10:15 a.m.**, the undersigned will make the following motion and tender the attached order.

**DEFENDANTS UofL HEALTH – LOUISVILLE, INC., UNIVERSITY OF
LOUISVILLE PHYSICIANS, INC., AND UofL HEALTH, INC.’S
MOTION FOR PROTECTIVE ORDER**

Defendants UofL Health-Louisville, Inc., University of Louisville Physicians, Inc., and UofL Health, Inc. (“UofL Defendants,” collectively), by counsel, respectfully request that the Court enter a Protective Order under CR 26.03. In support, the UofL Defendants state as follows:

INTRODUCTION

On May 22, 2026, at 11:42 p.m.—the Friday evening of Memorial Day weekend—Plaintiffs served an extraordinarily broad CR 30.02(6) Notice¹, which also

¹ Plaintiffs’ 30.02(6) Notice to take Videotaped Deposition, attached as **Exhibit 1**.

purports to be a live inspection protocol under CR 34, seeking testimony on eighteen (18) different topics from three (3) separate corporate defendants. The corporate representative deposition/live inspection was unilaterally noticed for June 4, 2026.

While the CR 30.02(6) Notice states that the June 4 date was selected for “efficiency” and that Plaintiffs may “adjust” the date, Plaintiffs did not even attempt to coordinate scheduling with defense counsel before unilaterally issuing the notice. The undersigned is familiar with Plaintiffs’ counsel’s discovery tactics in other litigation involving these same corporate defendants, including a recently tried matter that also involved expansive and technically complex discovery requests served on compressed timelines. The CR 30.02(6) Notice at issue reflects many of the same issues: unilateral CR 30.02(6) scheduling, voluminous eleventh-hour discovery requests, and an apparent attempt to force expedited compliance with unduly burdensome requests. Against this backdrop, the timing and breadth of the CR 30.02(6) Notice appear less directed toward the efficient advancement of discovery and more toward creating tactical pressure.

At any rate, what Plaintiffs seek is not merely corporate representative testimony regarding electronic medical record (“EMR”) processes. The CR 30.02(6) Notice seeks binding corporate testimony on highly technical and complex topics while simultaneously purporting to compel a live, forensic-style inspection of “ESI and any and all forms of data compilations from which information may be obtained[.]”² Plaintiffs’ desire to conduct a hybrid CR 30.02(6) deposition/live forensic

² Plaintiffs’ 30.02(6) Notice to take Videotaped Deposition, at p. 3.

inspection is procedurally improper, unduly burdensome, and creates a substantial risk of confusion. The UofL Defendants request that the Court prevent Plaintiffs from conducting a “fishing expedition” disguised as discovery. *Lexington Herald-Leader Co. v. Beard*, 690 S.W.2d 374, 379 (Ky. 1984).

ARGUMENT

CR 26.03 vests Kentucky courts with discretion to enter a protective order to preclude or limit discovery where “justice requires to protect a party from annoyance, embarrassment, oppression, or undue burden or expense.” CR 26.03. Courts may limit discovery if the discovery is (1) unreasonably cumulative or duplicative, (2) annoying, harassing, embarrassing, or oppressive, (3) obtainable from another source that is more convenient, less burdensome, or less expensive, or (4) the burden or expense of the proposed discovery outweighs its likely benefit. CR 26.03; *Ewing v. May*, 705 S.W.2d 910, 913 (Ky. 1986).

To protect the UofL Defendants from Plaintiffs’ oppressive and burdensome CR 30.02(6) Notice, the UofL Defendants request the Court enter a protective order: (1) limiting the scope of the corporate representative topics and (2) prohibiting Plaintiffs from conducting a live inspection of electronic systems. In the event the Court finds a live inspection is permissible, the UofL Defendants request that the Court bifurcate any CR 30.02(6) deposition(s) from Plaintiffs’ separate request for inspection under CR 34.

A. Plaintiffs’ CR 30.02(6) Notice seeks information beyond the permissible scope of discovery and fails to comply with CR 30.02(6)’s “reasonable particularity” requirement.

When issuing a deposition notice to a corporation under CR 30.02(6), the notice must “describe with reasonable particularity the matters on which examination is requested.” CR 30.02(6). Plaintiffs’ notice does the opposite. Rather than identifying discrete topics for examination, the notice seeks boundless testimony concerning system-wide EMR configuration, PACS integration, metadata, audit systems, patient portal functionality and other esoteric topics. The overbreadth of Plaintiffs’ CR 30.02(6) Notice is apparent on its face—it literally states that “the deposition may cover any topics within the general knowledge of the designee(s), not limited to the above-specific items.”³ By including this catchall provision, Plaintiffs effectively seek carte blanche access to examine a corporate designee on any topic they choose, regardless of whether it is enumerated in the notice. A corporate representative deposition is not a limitless inquiry into every conceivable area of corporate knowledge, and Plaintiffs’ approach is fundamentally incompatible with CR 30.02(6).

In addition to being extraordinarily broad, several topics improperly seek legal conclusions and privileged information. For purposes of this motion, the UofL Defendants will not belabor each and every objection to Plaintiffs’ CR 30.02(6) Notice. Rather, this motion is intended to provide the Court with examples of why a protective order is necessary to reasonably limit the scope of the topics and allow the UofL Defendants to meaningfully participate in narrowing and defining those topics.

For example, Topics 2-4 ask the UofL Defendants to “admit” or “contend” whether various individuals or patients had a particular legal “responsibility” or

³ Plaintiffs’ 30.02(6) Notice to take Videotaped Deposition, at p. 6.

“duty” with respect to communication of PET scan results, use of MyChart, and understanding the “clinical significance” of radiology findings.⁴ Similarly, Topics 11 and 14 ask whether the UofL Defendants “contend” that Dr. Gettis read the August 2022 PET scan report and whether Dr. Aria had a responsibility to communicate the 2022 PET scan report “at any time to anyone other than as was done,” in addition to demanding identification and production of any documents supporting such “contentions.”⁵ These topics far exceed the bounds of requests for corporate factual knowledge.

Plaintiffs cannot use CR 30.02(6) as a vehicle to force the UofL Defendants into providing binding corporate testimony on disputed legal duties and ultimate liability questions. Nor can it be used as a means of compelling the UofL Defendants to articulate litigation positions, which necessarily constitutes an invasion of counsel’s mental impressions and forces disclosure of the evidentiary bases for disputed claims and defenses. To the extent Plaintiffs seek the UofL Defendants to designate witnesses to testify on legal theories concerning standards of care, patient duties, or clinical responsibilities of providers, those matters are more appropriately addressed through expert testimony and trial proof—not through an open-ended corporate representative deposition.

B. The CR 30.02(6) Notice is an improper attempt to circumvent the procedures of CR 34.02.

⁴ Plaintiffs’ 30.02(6) Notice to take Videotaped Deposition, at pp. 3-4.

⁵ *Id.* at pp. 5-6.

Plaintiffs' CR 30.02(6) Notice contains the following demand for a live inspection of the UofL Defendants' EMR systems:

Each Deponent is requested to provide where relevant and/or material on any Topic, live access to all EHRs relating to Thelma ("Timmi") Tinker in NATIVE ELECTRONIC FORMAT, and in that regard **PLEASE BE ADVISED THAT THIS NOTICE IS ALSO A REQUEST BY PLAINTIFFS TO EACH DEPONENT PURSUANT TO CR 34.01(a)** to inspect and copy all any [sic] documents, materials, and EHRs, including but not limited to ESI and any and all forms of data compilations from which information may be obtained, and as more broadly defined at Appendix A. (emphasis in original).

CR 30.02(5) states that a deposition notice "may be accompanied by a request made in compliance with Rule 34 for the production of documents and tangible things at the taking of the deposition. The procedure of Rule 34.02 shall apply to the request." CR 30.02(5). In turn, Rule 34.02(2) provides that "[t]he party upon whom the request is served shall serve a written response within 30 days after the service of the request[.]" CR 34.02(2) (emphasis added).

Plaintiffs' CR 30.02(6) Notice disregards the procedural safeguards of Kentucky's civil rules. CR 34.02(2) expressly affords the UofL Defendants 30 days to respond to requests for inspection and assert appropriate objections. By unilaterally scheduling a live inspection of the UofL Defendants' EMR systems for June 4, 2026, Plaintiffs have deprived the UofL Defendants of the 30-day response period enumerated in CR 34.02(2). Plaintiffs cannot attempt to obtain quicker access to complex electronic systems simply by embedding a sweeping CR 34 request within a CR 30.02(6) deposition notice.

C. The Court should enter a protective order prohibiting Plaintiffs from conducting a live inspection of the UofL Defendants' electronic systems.

When a request for inspection is made under CR 34, it “shall specify a reasonable time, place, and manner of making the inspection and performing the related acts.” CR 34.02. Plaintiffs’ CR 30.02(6) Notice fails to satisfy any of these requirements.

Plaintiffs desire to conduct a simultaneous 30.02(6) deposition/live forensic-style inspection “via Zoom for use of its screen-sharing capability.”⁶ Plaintiffs further demand that a “live display from any EHR platforms as described below to be inspected pursuant to this notice, shall be displayed by the designee(s) and captured by screenshots from the Zoom recording of the electronic systems.”⁷ In addition, several of the topics direct the 30.02(6) witness to “explain and demonstrate” electronic navigation through various EMR systems. These Requests extend well beyond what is necessary to “inspect and copy” Tinker’s medical records as contemplated by CR 34.01 and would require the UofL Defendants to expose proprietary internal systems that have no bearing on the care at issue in this case.

Plaintiffs’ CR 34 request is extraordinarily intrusive. They seek direct, unrestricted, real-time remote access to the UofL Defendants’ live EMR systems. Permitting such access would certainly create significant privacy, security, and operational concerns. The risk of inadvertent disclosure of non-discoverable information—including HIPAA protected patient information relating to dozens of uninterested individuals—is not merely substantial, but virtually inevitable in the context of the live system access Plaintiffs seek. Moreover, a live inspection creates a

⁶ Plaintiffs’ 30.02(6) Notice to take Videotaped Deposition, at p. 2.

⁷ *Id.* at pp. 2-3.

significant risk of disrupting active healthcare operations and unnecessarily compromises the security of the UofL Defendants' network by increasing exposure to cyber security threats and unauthorized access.

These are not hypothetical concerns. Indeed, “[c]ourts have been cautious in requiring the mirror imaging of computers where the request is extremely broad in nature and the connection between the computers and the claims in the lawsuit are unduly vague or unsubstantiated in nature.” *John B. v. Goetz*, 531 F.3d 448, 459–60 (6th Cir. 2008) (quoting *Balboa Threadworks, Inc. v. Stucky*, No. 05-1157-JTM-DWB, 2006 WL 763668, at *3 (D. Kan. March 24, 2006)). “[T]he mere imaging of the media, in and of itself, raises privacy and confidentiality concerns” and “[d]uplication, by its very nature, increases the risk of improper exposure, whether purposeful or inadvertent.” *Goetz*, 531 F.3d at 437. Rule 34(a), “[l]ike the other discovery rules, ... allows the responding party to search his records to produce the required, relevant data. Rule 34(a) does not give the requesting party the right to conduct the actual search.” *In re Ford Motor Co.*, 345 F.3d 1315, 1317 (11th Cir. 2003).

In *Scotts Co. LLC v. Liberty Mut. Ins. Co.*, 2007 WL 1723509 (S.D. Ohio June 12, 2007), the plaintiff sought a forensic inspection of the defendants' computer systems based on a suspicion that certain discoverable information was being withheld. The Court declined to permit the inspection, reasoning that the plaintiff's “mere suspicion” and “speculation” was “entirely insufficient.” *Id.* at *2. Here, Plaintiffs fail to offer *any* basis, let alone a sufficient basis, as to why they are entitled to conduct a live inspection of the UofL Defendants' EMR systems. The UofL

Defendants have already produced relevant medical records and audit materials. To the extent Plaintiffs seek clarification regarding certain audit entries or system functionality, those matters may be explored through targeted written discovery requests. Plaintiffs are not entitled to access the UofL Defendants' live EMR environment simply because they requested a live inspection in a 30.02(6) notice.

Authorities addressing electronic stored information ("ESI") discovery likewise recognize that direct access to an opposing party's electronic systems is generally disfavored. The Sedona Conference, a nonpartisan legal think tank focused on data security and privacy law, published a set of core principles and best practice recommendations for addressing ESI disputes in federal or state court. *The Sedona Principles*⁸ are frequently cited by state and federal courts across the United States as persuasive authority regarding proportionality in ESI discovery. According to *The Sedona Principles*, "[i]nspection of an opposing party's computer system under Rule 34 and state equivalents is the exception and not the rule for discovery of ESI."⁹ In addition, *The Sedona Principles* caution that "Rule 34 inspections of ESI are likely to be particularly ineffective."¹⁰

CR 26.03 authorizes the Court, for good cause shown, to enter protective orders to protect a party from "annoyance, embarrassment, oppression, or undue burden or

⁸ *The Sedona Principles* were promulgated by the first Working Group on Electronic Document Retention Production, which consisted of attorneys and other individuals experienced in handling electronic information in the context of litigation. The First Edition of *The Sedona Principles* was published in 2004, followed by the Second Edition in 2007. The Third Edition of *The Sedona Principles* was most recently published in 2018.

⁹ *The Sedona Principles*, Third Edition: Best Practices, Recommendations & Principles for Addressing Electronic Document Production, 19 Sedona Conf. J. 1 (2018), Comment 6.d., attached as **Exhibit 2**.

¹⁰ *Id.*

expense.” CR 26.03. Good cause plainly exists here. Plaintiffs’ requested live inspection would be extremely burdensome. It would require coordination amongst IT staff and compliance personnel across multiple entities. It would also require preparation of live system access, real time oversight to prevent inadvertent disclosure of protected or irrelevant information, and consent from third-party EMR vendors.

To safeguard privacy and protect the UofL Defendants from Plaintiffs’ oppressive CR 34 request, the UofL Defendants requests the Court enter a protective order to protect them from the undue intrusiveness associated with Plaintiffs’ requested live inspection. *See* Fed.R.Civ.P. 34(a) Advisory Committee Note (2006) (recognizing that courts should “guard against undue intrusiveness resulting from inspecting or testing [electronic health] systems.”).

D. Plaintiffs’ attempt to combine a CR 30.02(6) deposition with a live inspection of electronic health systems is procedurally improper and unduly burdensome.

Plaintiffs’ CR 30.02(6) Notice improperly attempts to merge two fundamentally distinct discovery mechanisms. A CR 30.02(6) deposition and a CR 34 live inspection serve different purposes and implicate different concerns. The purpose of a corporate representative deposition is to obtain testimony concerning information known or reasonably available to the organization through a prepared witness. A CR 30.02(6) deposition is not a vehicle to conduct a real time walkthrough of the UofL Defendants’ proprietary electronic systems or provide live and unrestricted access to dynamic software.

A live inspection of various EMR systems utilized by the UofL Defendants implicates issues far beyond witness preparation, including HIPAA compliance, third-party information, system security, technical feasibility, and functionality of proprietary software. A corporate representative cannot be reasonably expected to provide binding corporate testimony while simultaneously navigating and demonstrating the functionality of various EMR systems. This invites confusion, inaccuracies, and disputes about system navigation. It is more appropriate for a corporate representative to testify about how a specific EMR system generally functions without conducting a live demonstration of the systems themselves. Requiring the UofL Defendants to do both at once conflates two distinct discovery mechanisms and creates substantial prejudice.

In the event the Court finds that Plaintiffs are entitled to conduct a live inspection of the UofL Defendants' EMR systems, the UofL Defendants request that the Court bifurcate any EMR inspection from any corporate representative deposition.

CONCLUSION

Based on the foregoing, the UofL Defendants ask this Court to enter a protective order limiting the scope of topics enumerated in Plaintiffs' CR 30.02(6) Notice and prohibiting Plaintiffs from conducting a live inspection of electronic systems.

Respectfully submitted,

/s/ Eleanor M.B. Davis

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CERTIFICATE OF SERVICE

I hereby certify that a true and correct copy of the foregoing has been served via the Court's electronic filing system and/or email this 28th day of May, 2026 upon the following:

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Chancellor R. Peterson

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Inc.; and UofL Health, Inc.*

EXHIBIT 1

NO. 25-CI-002964

JEFFERSON CIRCUIT COURT
DIVISION FOUR (4)
JUDGE JULIE KAELEN*ELECTRONICALLY FILED*CHARLES TINKER, INDIVIDUALLY AND
AS EXECUTOR OF THE ESTATE OF
THELMA TINKER, DECEASED, ET AL.

PLAINTIFFS

v.

**PLAINTIFFS' NOTICE TO TAKE VIDEOTAPE
DEPOSITION PURSUANT TO CR 30.02(6)**UofL HEALTH-LOUISVILLE, INC.
d/b/a UofL HEALTH, ET AL.

DEFENDANTS

* * * * *

Please take notice that Plaintiffs, by counsel, will, on **June 4, 2026**, at the hour of **10:00 a.m.**¹, local time, proceed to take the videotaped deposition of the corporate representative(s) designated by Defendants, UofL Health, Inc., UofL Health-Louisville, Inc. and University of Louisville Physicians, Inc. (each of which is separately a "Deponent" and each of which is required to identify the designee(s) binding it on each particular topic; all Deponents are, collectively, "UofL") at the office of Oldfather Law Firm, PLLC, 1330 South Third Street, Louisville, KY 40208.

Pursuant to CR 30.02(6), each Deponent is hereby advised of its duty to designate one or more persons who consent to testify on its behalf regarding knowledge reasonably available to Deponent on each Topic set forth below. It is

¹ For efficiency, this date was selected by the undersigned. If the date selected does not work for witness or counsel, please contact the undersigned, and we will adjust to a date within the court-ordered due dates.

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further the duty of each Deponent to fully prepare the designee(s) on the Topics for which the designee(s) are to testify on its behalf.

Plaintiffs will conduct the deposition via Zoom for use of its screen-sharing capability; all participants may elect to attend in person or remotely provided that remote access is available to designee(s) requiring access to EHR platforms as described below. Any designee(s) attending remotely will be sworn pursuant to KRS 423.325.

When the facts known or reasonably available to designee(s) are represented by documents, “identify and discuss” means to provide the Bates number(s) or the document(s) itself that reflect(s) the event or topic and to explain the manner in which the document(s) reflect(s) the event or topic at issue. When there is any uncertainty about the meaning of terms, please refer to Appendix A. Unless a different time frame is provided, the time frame for each topic is from the date of August 8, 2022 through to the present.²

Exhibits and other demonstrative evidence utilized by Plaintiffs shall wherever possible be displayed electronically to any remote designee(s). Exhibits and other demonstrative evidence utilized by the designee(s), specifically but not only “live” display from any EHR platforms as described below to be inspected pursuant to this notice, shall be displayed by the designee(s) and captured by screenshots from the

² Counsel are prepared to discuss and attempt to resolve any disagreement as to scope or content of the Topics; please contact the undersigned as soon as convenient should that be necessary.

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Zoom recording of the electronic systems. Where appropriate or requested, those screenshots may be marked as deposition exhibits.

Except objections pursuant to CR 30.02(4)(e) which shall be stated succinctly, objections are reserved and shall not be stated on the video recording.

Each Deponent is request to provide where relevant and/or material on any Topic, live access to all EHRs relating to Thelma (“Timmi”) Tinker in NATIVE ELECTRONIC FORMAT, and in that regard **PLEASE BE ADVISED THAT THIS NOTICE IS ALSO A REQUEST BY PLAINTIFFS TO EACH DEPONENT PURSUANT TO CR 34.01(a)** to inspect and copy all any documents, materials, and EHRs, including but not limited to ESI and any and all forms of data compilations from which information may be obtained, and as more broadly defined at Appendix A.

Each Deponent shall provide all knowledge reasonably available to Deponent on these Topics:

1. With regard to Timmi’s MYCHART, please explain and demonstrate in Timmi’s MYCHART:
 - a) How access to the August 15, 2022 PET scan report was provided through Timmi’s MYCHART;
 - b) When access to the August 15, 2022 PET scan report was provided through Timmi’s MYCHART;
 - c) Whether Deponent can establish Timmi read the August 15, 2022 PET scan report on her MYCHART and if so, when, by whom and please demonstrate how this is known;
 - d) Whether Deponent can establish that the August 15, 2022 PET scan report was printed from Timmi’s MYCHART and if so, when, by whom, to what device, and please demonstrate how this is known;
2. Does Deponent admit that it was Dr. Gettis’ responsibility to report to Timmi directly the results of the August 15, 2022 PET scan report?
3. Does Deponent contend that patients have an established standard of care to:

EXHIBIT 1

- a) Use the MYCHART functionality;
 - b) Be aware of all test results posted in an available MYCHART;
 - c) Understand the clinical significance of a PET scan report.
4. Does Deponent contend that Timmi had a duty to:
 - a) Use the MYCHART functionality;
 - b) Be aware of all test results posted in her MYCHART;
 - c) Understand the clinical significance of the August 15, 2022 PET scan report.
5. With regard to Deponent's PACS³, please explain and demonstrate how the original imaging and report of Timmi's August 15, 2022 PET scan report migrated from the PACS to Timmi's MYCHART.
6. Describe the In-Basket system, including how messages are generated, tracked, routed, received, opened, acknowledged, completed, and audited.
7. Describe the "acknowledge[d]" function in UofL's EHR and AUDIT DATA, including its relationship to provider workflow for receipt, review, and acknowledgment of test and radiology results.
8. Describe UofL's In-Basket configuration for moving, retaining, or clearing "acknowledged" results or messages.
9. Describe whether, and if so how, the "acknowledgment" function affects release of test and/or radiology results to MyChart.
10. With regard to Deponent's PACS⁴, please explain and demonstrate how the original imaging and report of Timmi's August 15, 2022 PET scan report migrated from the PACS to Dr. Gettis' In-Basket.

³ This is a reference to the Picture Archiving and Communication System (PACS) used by Deponent's radiology services as medical imaging technology that provides filmless storage, retrieval, management, and distribution of digital medical images, specifically here Timmi's August 15, 2022 PET scan report/images.

⁴ This is a reference to the Picture Archiving and Communication System (PACS) used by Deponent's radiology services as medical imaging technology that provides filmless storage, retrieval, management, and distribution of digital medical images, specifically here Timmi's August 15, 2022 PET scan report/images.

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11. Does Deponent contend that Dr. Gettis read the August 15, 2022 PET scan report at any time?
 - a) If so, when?
 - b) Is this contention supported by any document?
 - c) If so, please identify and provide such document(s).
12. Describe and explain the produced AUDIT DATA, including how it was generated, how fields were selected for production, and the extent to which the following are absent:
 - a) MyChart message content;
 - b) In-Basket AUDIT DATA captured by UofL's EHR but not produced;
 - c) In-Basket message content;
 - d) IP address data;
 - e) AUDIT DATA reflecting patient-user access to, viewing of, or review of test and/or radiology results;
 - f) URLs associated with web page entries;
 - g) Record-specific logging identifying, for example, the particular test or radiology records accessed, viewed, or printed;
 - h) Print-related details, including PDF exports, page counts, printer destinations, and the specific documents printed;
 - i) Image-viewer links or integrations to radiology imaging;
 - j) Logging showing when test and/or radiology results were released to the patient;
 - k) Patient notifications, including when they were sent and whether they were opened; and
 - l) Documented communication or follow-up concerning Ms. Tinker's PET scan results in August 2022.
13. Describe UofL's policies and procedures for maintaining all AUDIT DATA and other information contained in its EHR systems, including but not limited to COMMUNICATIONS, from January 1, 2022, to the present;

EXHIBIT 1

14. Does Deponent contend that Dr. Aria had a responsibility to communicate the August 15, 2022 PET scan report results at any time to anyone other than as was done?
 - a) If so, why?
 - b) Is this contention supported by any document?
 - c) If so, please identify and provide such document(s).
15. For each EHR used by UofL from January 1, 2022, to the present, produce original exports, copies, and screenshots of all MEDICAL RECORDS relating to Ms. Tinker, including all PHI and all AUDIT DATA, as directed during the deposition and inspection.
16. Produce any key, legend, data dictionary, code table, or other reference material necessary to interpret the AUDIT DATA.
17. Describe whether each Deponent is aware of any documents that have been withheld under any other claim of privilege.
18. Produce in native format and discuss: all documents received, prepared, reviewed, referenced, and/or relied upon, or supportive of any and all of the foregoing topics and of designee's preparation for and/or testimony herein; most current copy of each designee's *curriculum vitae*; each designee's complete job description; and all records of whatever nature (including texts, emails, excel files, and all digital and paper documents) pertaining to Timmi, by whomever generated, that are or have been in each designee's possession, which have been reviewed by each designee, or are or have been subject to any Deponent's control.

Please be further advised that the deposition may cover any topics within the general knowledge of the designee(s), not limited to the above-specific items.

Designee(s) should be ready to discuss and describe all efforts made to search for, locate, and produce the documents responsive to any of the Topics set forth herein, as well as to produce and discuss all documents pertaining to, referenced by, identified in, or supportive of all the foregoing Topics.

The deposition shall continue from day to day until completed, excluding Sundays and holidays. It will be recorded stenographically and by video and may be

EXHIBIT 1

used for discovery and for all other purposes permitted by the Kentucky Rules of Civil Procedure.

Respectfully submitted,

/s/ Michael R. Hasken

Ann B. Oldfather (KBA 52553)

Sheldon L. Haden (KBA 88876)

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7/07/2026 09:00:00 AM 25-CI-002964-000007-000007-363916672839B : 000007 of 000012

EXHIBIT : 000007 of 000012

EXHIBIT 1

CERTIFICATE OF SERVICE

The above signature certifies that, on May 22, 2026, the foregoing was electronically filed with the Clerk of Court using the KCOJ e-filing system and was served via email, to:

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APPENDIX A: DEFINITIONS

A. Hereafter, “UofL” refers generally to the Defendants as defined in Plaintiffs’ Complaint, including all healthcare facilities owned and/or operated by UofL that provided medical care to Ms. Tinker, including but not limited to care provided by Dr. Erica Gettis, D.O., and Robert Aria, M.D. “UofL” also includes all agents, affiliates, predecessor entities, acquired entities, related entities, brother-sister corporations, parent-subsidary corporations, and any other entity acting or purporting to act on UofL’s behalf or directing UofL to act for its benefit.

B. Whenever the term “DOCUMENT(S)” is used herein, it includes all written, printed, typed, word-processed, copied, photographed, recorded, audio, video, or computer-generated materials, including all electronically stored information (“ESI”). This definition includes metadata (including audit data), and all versions, drafts, revisions, and duplicates. It also includes all COMMUNICATIONS, as defined below, all DOCUMENTS maintained, stored, or generated on any electronic transfer or storage system, information generated by collaboration and communication platforms, and all other writings in UofL’s possession, custody, or control (including UofL’s right to access or obtain such DOCUMENTS upon demand).

C. Whenever the term “COMMUNICATION(S)” is used herein, it means any disclosure, transfer, or exchange of information. This includes, without limitation, oral and electronic communications such as face-to-face conversations, teleconferences, telephone calls, mail, messaging, chat, facsimile, videoconferencing, and personal delivery. The term also includes information

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shared on collaboration and communication platforms, as well as comments and communications within shared or tagged documents such as Microsoft Word documents, spreadsheets, and PowerPoint presentations. If a COMMUNICATION is part of, attached to, or linked with a DOCUMENT, it falls within the definition of DOCUMENT above. If a COMMUNICATION includes hyperlinks or other pointers to attachments, or has a document attached to it, both the COMMUNICATION and the attached or linked DOCUMENT are included within the definition of DOCUMENT.

D. Whenever the term “PERSON” is used herein, it refers, without limitation, to a natural person, as well as any corporate, business, and/or governmental entity.

E. Whenever the term “AUDIT DATA” is used herein, it refers to: “complete historical record of entries regarding patient care information automatically collected and stored by electronic health records (EHR) software or, in the case of paper records, collected and stored as a matter of industry standard and related policy and procedure... information entered or altered (changed or deleted) by users or processes and information concerning all users who accessed or who made, changed, or caused entries to be made into the EHR or paper medical record. In the case of EHR, this collection includes, but is not limited to, information regarding demographic data about the user and facts about access and actions taken by the user, such as date, time, location, and area of record accessed/actions taken, and the actions taken by the user or process in the record, such as creation, queries, views, additions, modifications, deletions, and so forth.”

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E2147-18 at ¶¶ 3.1.3-3.1.3.1. Audit data can be aggregated into reports, commonly called “audit logs” or “audit trails.” *Id.* at ¶ 3.1.9.1. E2147-18 is incorporated by reference under 45 CFR § 170.299(c)(1).

F. “HEALTHCARE PROVIDER” shall refer to all UofL employees, agents, and consulting providers who treated, cared for, or rendered services to Ms. Tinker.

G. “NATIVE ELECTRONIC FORMAT” shall refer to the format in which ESI was generated, or the format used by the producing party in the ordinary course of business, with the file’s original metadata intact, meaning the metadata fields have not been altered, deleted, updated, or otherwise modified.

H. “ESI” shall refer to electronically stored information, including data and DOCUMENT(S) stored in any electronic media or format.

I. “METADATA” shall refer to: (a) information embedded in or associated with a native file; (b) information generated automatically by the operation of a computer or other information technology system when a native DOCUMENT is created, edited, modified, transmitted, deleted, and/or otherwise manipulated by a user; (c) information about a record in a document-management system, whether created by a user or generated by the system; and (d) information collected during the collection of DOCUMENTS, such as the name of the storage media device on which a document was stored, or the custodian or non-custodial source from which it was collected. METADATA is a subset of ESI, and all requests for DOCUMENTS therefore include all METADATA relating to those DOCUMENTS.

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J. “Electronic Medical Record” (“EMR”) or “Electronic Health Record” (“EHR”) means any ESI containing Protective Health Information (“PHI”) for Plaintiff. EMR and EHR are used interchangeably and are defined by HIPAA, 42 U.S.C. 1320d-2 to 1320d-9, and accompanying regulations, 45 C.F.R. Parts 160 and 164, as well as the Standard Specification for Audit Disclosure Logs for Use in Health Information Systems – ASTM E2147-18, which is incorporated under 45 CFR 170.299.

K. “MEDICAL RECORDS” shall include all EHR and/or other ESI, paper records, written recordings, and media containing PHI of Ms. Tinker. “The term ‘record’ means any item, collection, or grouping of information that includes protected health information and is maintained, collected, used, or disseminated by or for a covered entity.”¹

L. “PHI” means protected health information.

¹ 45 CFR 164.501(2).

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The Sedona Conference Journal

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2018

The Sedona Principles, Third Edition: Best Practices, Recommendations & Principles for Addressing Electronic Document Production

The Sedona Conference



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THE SEDONA PRINCIPLES, THIRD EDITION:
BEST PRACTICES, RECOMMENDATIONS & PRINCIPLES FOR
ADDRESSING ELECTRONIC DOCUMENT PRODUCTION

*A Project of The Sedona Conference Working Group on
Electronic Document Retention and Production (WG1)*

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procedures employed to preserve, collect, and prepare the materials for production; and the steps taken to ensure the integrity of the information throughout the process. Since *The Sedona Principles* was first published, applications have been developed to automate the legal hold issuance, tracking, and documentation processes, as well as some collections. Having documentation can help respond to legitimate challenges (*see* Comment 6.b.)—even those made years later—to the processes employed, avoid overlooking ESI that should be collected, and avoid collecting ESI that is neither relevant nor responsive to the matter at issue. Organizations should endeavor to revise their standardized documentation and validation procedures as appropriate, e.g., when the organization introduces new technologies to store or create ESI, including some technologies that create new types of ESI.

Parties may voluntarily agree to produce or exchange documentation of their discovery processes, but should do so only after due consideration of privilege and work product issues (*see* Principle 10). Absent such a voluntary agreement, however, parties should not be required to produce documentation of their discovery processes unless there has been a showing of a specific deficiency in their discovery processes (*see* Comment 6.b.), and, even then, the production requirement is subject to the normal protections afforded by the attorney-client privilege or the work product doctrine.

Comment 6.d. Rule 34 inspections of electronic information systems are disfavored.

Courts have repeatedly found that Rule 34 does not create a routine right of direct access to an opposing party's electronic

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information system.¹⁰⁶ Inspection of an opposing party's computer system under Rule 34 and state equivalents is the exception and not the rule for discovery of ESI. In the majority of cases, the issues in litigation relate to the informational content of the data stored on computer systems, not the actual operations of the systems; and, as noted above, the obligation to produce relevant content lies with the responding party. Unless the requesting party can prove that the actual operation of a particular system is at issue in the litigation, if the responding party provides the informational content of the data, there is no need or justification for direct inspection of the responding party's computer systems.

Direct access to an opposing party's computer systems under a Rule 34 inspection also presents possible concerns such as:

- a) revealing trade secrets;
- b) revealing other highly confidential or private information, such as personnel evaluations and payroll information, properly private to individual employees;
- c) revealing confidential attorney-client or work-product communications;
- d) unreasonably disrupting the ongoing business;
- e) endangering the stability of operating systems, software applications, and electronic files if certain procedures or software are used inappropriately; and

106. See, e.g., SEC v. Strauss, No. 09 Civ. 4150, 2009 WL 3459204, at *12 n.8 (S.D.N.Y. Oct. 28, 2009) ("There is a general reluctance to allow a party to access its adversary's *own* database directly. The Advisory Committee Notes to the 2006 amendments to Rule 34 explain that Rule 34(a) is not meant to 'create a routine right of direct access to a party's electronic information system' and advises that courts 'guard against undue intrusiveness resulting from inspecting or testing such systems.' Thus, courts have declined to find an automatic entitlement to access an adversary's database." (citing cases) (emphasis in original)).

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- f) placing a responding party's computing systems at risk of a data security breach.

Further, Rule 34 inspections of ESI are likely to be particularly ineffective. The typical production, in which the responding party identifies and produces responsive information, allows the party with the greatest knowledge of the computer systems to search and utilize the systems to produce responsive information. A Rule 34 inspection, in contrast, requires persons unfamiliar with the party's recordkeeping systems, hardware, and software to attempt to manipulate the systems. Not only is such a process disruptive, it is less likely to be fruitful. In most cases, responding parties will be able to argue persuasively that its production of relevant ESI from computer systems and databases was sufficient to discharge its discovery obligations.

To justify the onsite inspection of a responding party's computer systems, a requesting party must demonstrate that there is a substantial need to discover information about the computer system and programs used (as opposed to the data stored on that system) and that there is no reasonable alternative to an on-site inspection. Any inspection procedure should: (1) be documented in an agreed-upon (and/or court-ordered) protocol; (2) recognize the rights of non-parties, such as employees, patients, and other entities; and (3) be narrowly restricted to protect confidential and personally identifiable information and system integrity as well as to avoid giving the discovering party access to information unrelated to the litigation. Further, courts that have allowed access generally have required that the inspection be performed by a qualified consultant or vendor, and that no information obtained through the inspection be produced until the responding party has had a fair opportunity to review that information.

CASE NO. 25-CI-002964

JEFFERSON CIRCUIT COURT
DIVISION FOUR (4)
JUDGE JULIE KAE LIN

CHARLIE TINKER, Individually and as Executor
of the ESTATE OF THELMA TINKER, Deceased,
and DENISE HOUCK, MONICA RUSSELL, BRIAN
TINKER, AND CHERI KALILIKANI, Each Individually

PLAINTIFFS

v.

UofL HEALTH–LOUISVILLE, INC. d/b/a UofL HEALTH;
UNIVERSITY OF LOUISVILLE PHYSICIANS, INC.;
ERICA GETTIS, D.O.; ROBERT ARIA, M.D.; and
UofL HEALTH, INC.

DEFENDANTS

ORDER

This matter is before the Court upon Defendants UofL Health-Louisville, Inc., University of Louisville Physicians, Inc., and UofL Health, Inc.'s Motion for Protective Order. The Court, having reviewed the pleadings, heard arguments, and being sufficiently advised:

IT IS HEREBY ORDERED AND ADJUDGED that:

1. Defendants' Motion for Protective Order is **GRANTED**.
2. Plaintiffs are prohibited from conducting a live inspection of Defendants' electronic health systems.
3. Plaintiffs' 30.02(6) deposition topics shall be limited to matters described with reasonable particularity and shall not include:
 - a. Requests for legal conclusions or legal contentions;
 - b. Requests seeking attorney-client privileged communications;
 - c. Requests seeking attorney work product; or

- d. Requests requiring disclosure of litigation strategy.
4. Should Plaintiffs elect to issue an amended 30.02(6) notice, they shall serve a separate 30.02(6) notice on each corporate defendant from whom they seek testimony.
5. Should Plaintiffs elect to issue an amended 30.02(6) notice, the parties shall meet and confer in good faith regarding narrowed topics. Plaintiffs shall also confer with defense counsel to select a mutually agreeable date for any 30.02(6) deposition(s) prior to issuing such notice(s).

SO ORDERED this ____ day of _____, 2026.

HON. JULIE KAE LIN
JUDGE, JEFFERSON CIRCUIT COURT
DIVISION FOUR (4)

TENDERED BY:

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